

Daily Update — Dad's care, 2026-05-19

AT A GLANCE

- **First IV ketamine session at Royal Oak yesterday (5/18).** Session itself was difficult — shaking, crying out. Mom: "terrible, convulsive."
- **Afterward Dad was more coherent than Mom has seen him in a long time** — clear, measured, articulate. Dad himself said he didn't feel better.
- **Dad's framing after the Julane call:** didn't feel attached to anyone during the session; whole experience felt like a runaway train. Julane: ketamine can be a door, but Dad wasn't in a position to walk through it this time. February's experience felt supported (family around him); this time he felt totally alone.
- **Chilton's framing of ketamine:** an intensifier of the emotions you bring into the session.
- **Royal Oak: off the table.**
- **Ann Arbor clinic start: revised to June 8** (was tentatively early June / June 1). Chilton pushed it back to allow the Cymbalta → Lexapro switch to complete first; her stated reasoning is that she considers starting ketamine on Cymbalta riskier than on Lexapro.
- **CA option:** off the table per Dad.
- **Dad confirmed he did not take the Seroquel** that was proposed last weekend.
- **Caffeine update (correction):** Dad has been drinking only one cup with breakfast, nothing after. Coffee had been stirring tension. Mom's prior report was accurate.
- **Sleep state:** one nocturia trip per night, no dream recall, wakes unrested, cannot nap during the day.
- **Memory in today's calls:** spotty and hesitant — Dad didn't initially recall a call from ~1 hour earlier. Less impaired than during the San Francisco period in February.
- **Provider sequencing differences are now clearer:** Chilton wants the SSRI switch completed before ketamine starts and reportedly mentioned Depakote (ambiguity below). Julane would sequence the other way — see how the first 1–2 ketamine sessions go before initiating any Cymbalta withdrawal. Rubin call scheduled tomorrow (5/20) 11 AM.
- **Chilton expressed frustration with Dad for "overstepping bounds"** — her clinical role is narrower than the family conversation has been treating it.
- **Dad's emotional state:** dejected; explicitly asked the family to maintain genuine (not false) hopefulness.

WHAT WE KNOW

- **5/18 session at Royal Oak:** shaking, crying out, prolonged distress. Dad described the commute as "going to Africa." No provider in the room during the session.
- **Post-session functional state on 5/18:** "more coherent than Mom has seen him in ages." Dad said he didn't feel better.
- **Dad's processed experience after Julane (5/19):** didn't feel attached to anyone; runaway train; not in a position to walk through the door ketamine was opening; felt alone in a way February's IV course did not.
- **February 2026 ketamine course was IV and produced a dramatic response.** Later lozenge use was good but less transformative. Dad is specifically an IV-ketamine responder. The Feb context included family presence and a felt sense of support — a structural difference from 5/18 worth holding onto.
- **Dad did not take the proposed quetiapine.** See [handout #18](#).
- **A2 ketamine start: June 8.** Chilton's stated rationale: allow Cymbalta → Lexapro switch to complete first.

- **Current sleep stack:** doxepin 6 mg HS, trazodone 50 mg PRN HS, melatonin 3 mg HS. See [handout #13](#) and [handout #19](#).
- **Current antidepressant:** duloxetine 60 mg, on board since 11/2025 (~6.5 months).
- **RBD episode frequency trajectory:** episodes have decreased over the past year; most recent ~February 2026, now roughly every few months. No dream recall reported recently.
- **Sleep pattern detail:** one nocturia trip per night (down from prior pattern), no dream recall, wakes unrested, cannot nap.
- **Caffeine intake actual:** one cup with breakfast, nothing after.
- **Cognitive in-call observation (5/19):** Dad's memory was spotty and hesitant in real time — at one point he didn't initially recall a call from ~1 hour earlier. Less impaired than during the San Francisco period in February.

WHAT WE DON'T KNOW YET

- **Whether Chilton specifically "recommended" Depakote** or simply noted it was on the chart. Dad's memory of the call is hesitant; Mom recalls Dad saying she recommended it. No written record yet.
- **What other recommendations Chilton may have made** that Dad doesn't remember and didn't note.
- **Exactly what Dr. Rubin is recommending** — Rubin call tomorrow (5/20) at 11 AM.
- **Whether Chilton's concern about Cymbalta + ketamine concurrent use is supported by current evidence.** Del Casale 2025 (JAMA Psychiatry, n=55,480) found that SNRI + esketamine outperformed SSRI + esketamine on mortality, hospitalization, and relapse. The Feb 2026 IV course was on duloxetine background. Worth direct comparison.
- **Whether the post-session 5/18 coherence persisted overnight**, or faded.
- **What an Ann Arbor clinic intake looks like operationally** — protocol, room arrangement, in-room provider presence, pre-session anxiolysis protocol.
- **Whether Dad is open to a single pre-session benzodiazepine dose** as part of the A2 intake.
- **Whether the RBD-episode-frequency trajectory (decreasing over recent months)** materially modifies the prior probability of underlying synucleinopathy progression — and therefore the risk math on the agents we've been conservative about. This is queued as an OpenEvidence question.

FOLLOW-UPS NEEDED

1. **Capture call notes after each provider conversation going forward.** Mom takes notes after each call (and ideally Dad too where feasible). Memory is uneven enough that without notes, ambiguities like "did Chilton recommend Depakote or just mention it" can't be resolved later.
2. **Tomorrow's 11 AM Rubin call:** capture (a) his specific written recommendation, (b) the indication for each proposed agent (especially Depakote), (c) the proposed timing relative to the June 8 ketamine start, (d) his read on Julane's preferred sequencing.
3. **Email Chilton directly** to confirm in writing: *"Did you specifically recommend divalproex (Depakote), or did you note it was on his chart? If recommended, for what indication?"* Same email asks for the reasoning behind preferring Lexapro over Cymbalta as the ketamine background, given Del Casale 2025 and Dad's documented duloxetine + IV response.
4. **Get Julane's written read** of yesterday's session and her sequencing preference.
5. **Confirm A2 clinic intake details:** date (June 8), protocol, in-room provider, pre-session anxiolysis approach.

CONCERNS ABOUT THE PROPOSED PLAN

Cross-referenced to prior research where relevant.

- **The Cymbalta → Lexapro switch contradicts the stated goal of reducing acute pre-session anxiety.** Duloxetine discontinuation produces acute anxiety, dizziness, paresthesias, nausea, agitation. Lexapro start carries activation risk in the first 2–4 weeks, especially in older adults; Dad's [activation-intolerance pattern](#) makes him a worse-than-average candidate. SSRI anxiolytic effect takes 4–8 weeks — even with the June 8 start, the turbulence window lands before any benefit window.

- **Chilton's "SNRI is riskier than SSRI as a ketamine background" framing is not the standard read.** Del Casale 2025 (JAMA Psychiatry, n=55,480) found the opposite at population scale: SNRI + esketamine outperformed SSRI + esketamine on mortality (5.3% vs 9.1%), hospitalization, and relapse. Dad's documented prior IV response was specifically on duloxetine. Worth asking Chilton what she's basing the framing on. (Queued as OE re-evaluation.) See also [handout #06](#), which will be revisited.
- **Julane's preferred sequencing is structurally more conservative.** Run the first 1–2 ketamine sessions on the existing stack, observe response and tolerability, then revisit any antidepressant switch with that data in hand. Advantage: doesn't introduce two pharmacological changes (SSRI switch + new modality) in the same window; preserves the duloxetine background the documented Feb 2026 response was on.
- **Removing trazodone and doxepin is the wrong direction.** Both are AGS-Beers-endorsed at these doses, Chilton-cleared, Dad-tolerated. See [handout #13](#). Lemborexant alone (Julane-endorsed) is reasonable as an addition per [handout #19](#); coupling it to removal of the working stack is a different and worse proposal.
- **Divalproex is the most ambiguous element of the plan.** Chilton's role is unclear: recommended, or merely noted on the chart. Dad's memory of the call is hesitant. Even if recommended, the indication isn't stated. Divalproex in older adults brings cognitive effects, sedation, weight gain (in someone already losing weight), hepatotoxicity, thrombocytopenia, and — most relevantly for Dad — a documented drug-induced parkinsonism signal that overlaps with the Lewy-spectrum question the neurology workup is meant to address. See [handout #01](#). **If divalproex does end up being pursued, the absolute family-side requirement is to surface any new motor or cognitive symptoms early** rather than mask or minimize them, so an unrecognized drug-induced parkinsonism doesn't get attributed to disease progression. (Queued as OE re-evaluation.)
- **The post-session coherence on 5/18 was a real ketamine response signal**, not a footnote. Dad's specific IV-responder status (Feb 2026) reinforces it. The session was destabilizing in the chair; the post-session functional state is a separate variable.
- **The Feb 2026 context vs. the 5/18 context is informative.** Feb: family present, sense of support, dramatic response. 5/18: Royal Oak, no in-room provider, "Africa" framing, didn't feel attached to anyone. Set and setting shape both experience and antidepressant response. The A2 transfer is the practical lever.

RECOMMENDED PATH

1. **Capture all provider communications in writing** going forward (Mom and/or Dad after each call). Memory is uneven; ambiguities compound.
2. **Confirm Chilton's actual recommendations in writing** — Cymbalta switch reasoning, Depakote (recommended or noted), any other items.
3. **Tomorrow's Rubin call (5/20, 11 AM):** ask for the specific written recommendation, indication for each proposed agent, proposed timing.
4. **Hold the medication stack stable through the gap** until the open clinical questions are resolved. Duloxetine, doxepin, trazodone PRN, melatonin continue. If Cymbalta → Lexapro proceeds, it should be on the basis of an explicit, evidence-supported indication, not a default.
5. **Lemborexant as a sleep addition is fine and Julane-endorsed.** Don't couple it to removal of doxepin/trazodone. Matches [handout #19](#).
6. **Defer the Depakote conversation** until (a) Chilton's recommendation is confirmed and indication is stated, and (b) the family is set up to surface any new motor or cognitive symptoms early. If it proceeds, weekly symptom check-ins with explicit motor and cognitive screening should be in place from the start.
7. **Use the 2–3 week gap before June 8 deliberately** for behavioral preparation with Julane and the NPI top-tier items from [handout #19](#). Caffeine is already at one cup AM; bright-light parameters, digital CBT-I, exercise to 3×/week, and alcohol abstinence remain on the list. The new sleep detail (no dream recall, unrested, can't nap) strengthens the case for prioritizing the PSG with RBD montage and OSA workup — that single study could clarify whether the unrested-on-7-hours pattern has an apnea component.

- 8. Plan A2 intake with explicit pre-session anxiolysis protocol** — single-dose lorazepam pre-session, in-room provider presence, attention to set/setting. The Feb 2026 supported-and-around-family context produced a dramatic response; the 5/18 alone-on-runaway-train context did not. The difference is the lever we have.

OPEN QUESTIONS QUEUED FOR OPENEVIDENCE (RE-EVALUATION)

- **SSRI vs SNRI as the antidepressant background for IV ketamine**, in light of Del Casale 2025 — directly tests Chilton's stated reasoning for the switch.
- **Divalproex in late-life depression with prodromal-synucleinopathy features** — risk of drug-induced parkinsonism, evidence base for the indication.
- **RBD episode frequency trajectory and its implications** for synucleinopathy-progression risk and for medication risk-tolerance — testing whether Dad's decreased episode frequency over recent months should modify the priors that drove conservative posture in handouts #17 and #18.

HANDOUTS LIKELY NEEDING REVISION AFTER THE OE RESPONSES

- [#06 Cymbalta → Lexapro](#) — now newly active; Chilton's stated reasoning warrants direct comparison to Del Casale 2025 and to Dad's documented duloxetine + IV ketamine prior response.
- [#01 Depakote](#) — re-evaluation in the context of Chilton's apparent recommendation (pending confirmation) and the RBD-frequency trajectory.
- [#17 Mirtazapine retrial](#) — RBD-related concerns may shift if the frequency trajectory reduces the prior probability of imminent synucleinopathy progression.
- [#18 Quetiapine low-dose](#) — DLB-spectrum risk framing also depends on the RBD-frequency question.

HOLDING ALONGSIDE THE CLINICAL *Dad has explicitly asked the family to maintain genuine hopefulness rather than false hope. The relevant framing is that recovery from severe TRD of this duration is a long, slow climb, not a single intervention. Ketamine, in the best of circumstances, was always going to be the first step on a longer path, not a shortcut to the top. The 5/18 session was a hard step; it does not change the path or the destination. The June 8 restart at A2 — with Julane present, in a familiar setting, with explicit pre-session preparation — is a meaningfully different context and the next step on the same path.*